https://aarhuswiki.dk/images/c/c4/Sindssygeanstalten.jpg; *Sindssygeanstalten for Nørrejylland i Risskov, **1859**. Fotograf: Axel Thorkil Cammer Svendsen, reproduktion Aarhus Kommunes Biblioteker, Lokalhistorisk Samling*

1) Introduction

Here you explain the purpose of your investigation and introduce your thesis or research question.
a) Problem Orientation and Motivation: *What is your research question? What (cultural, historical, technical) problem did you address? What motivated you?*

"Goodbye Yellow Brick Road" is a song written by British musician Elton John and lyricist Bernie Taupin". Since 2018, the psychiatric hospital, originally located in Risskov, has been transformed into a (), Bindebøll Byen.[1]
Yet not everyone was privileged enough to say goodbye. For some, life ended behind the old yellow-brick walls of the psychiatric hospital in Risskov.

Patient Records
Today, the surviving patient records offer a different point of entry into the institution and the lives documented within it. Their digitization has made historical material accessible beyond the physical archive; however, digital accessibility does not make archival material immediately available for computational historical analysis. In their digitized form, the records remain organized according to the structure of the historical registers and their subsequent transcription, requiring further computational transformation before patterns across thousands of individual entries can be examined. This presents both a historical and technical problem: how can these records be

[1] AarhusWiki, "Bindesbøll Byen," accessed August 17, 2026, https://aarhuswiki.dk/wiki/Bindesb%C3%B8ll_Byen.

transformed into analyzable data while remaining attentive to the historical and archival processes through which the data were produced?

Against this background, the present study asks:

****RESEARCH QUESTION HERE** (How did the psychiatric institution in Risskov make its patient population administratively legible between 1889 and 1913, and what patterns become visible when its digitized patient registers are examined computationally?)**

The project therefore investigates the classificatory practices embedded in the patient registors, with particular attention to the relationship between sex, psychiatric diagnosis, and institutional trajectory. At the same time, it reflects on the transformation of historical sources into computational data and the forms of archival mediation involved in this process. Digitization and transcription make distant analysis possible, but they do not provide an unmediated representation of the historical patient population: what can be analyzed remains conditioned by what the institution recorded, what the archive preserved, what was transcribed, and how those records are subsequently operationalized as data.

To understand the conceptualization and operationalization of the subsequent analysis - and its necessarily interpretive character - the following section historically contextualizes the institution and its patient registers. This provides the basis for the hypotheses, analytical choices, and interpretation of the findings that follow.

b) Background:
Provide enough background to enable the reader to understand the problem being investigated. ***Introduce its (historical, cultural, or technical) relevance and significance.*** *If you can, include a 'literature review' summarizing previous research addressing the same or a related problem. Define and critique the concepts and claims you use / reproduce, back up your statements with references.*

The Historical Background of Risskov Psykiatri

- *"Helbredelsesanstalten for Sindssyge i Nørrejylland, kaldet Jydske Asyl, der blev taget i brug i 1852 med Selmer som overlæge."* + *"Hospitalet her ved Århus blev grundlagt i 1852 på hans idéer og var det første ordentligt reglementerede behandlingshospital for sindssyge i kongeriget."*
- *"Før i tiden blev hospitalet i folkemunde kaldt "Jydske Asyl". Hospitalet skulle fungere som et tilflugtssted, hvor den syge kunne tilbydes "ro, renlighed og regelmæssighed"." (aarhuspanorama: https://aarhuspanorama.dk/livet-bag-lukkede-doere-psykiatrisk-hospital/)*
- *"Præget af tidens romantiske opfattelse blev hospitalet anlagt smukt ved skov og bugt - adskilt fra de skadelige påvirkninger fra storbyen Aarhus (ca. 10.000 indb.)."*
- *"Helbredelsesoptimismen var dog for stor, og i 1861 udbyggede man hospitalet med en større plejeafdeling for uhelbredelige. Disse uhelbrede-lige patienter kunne så tilbringe mange år – måske resten af deres liv – i et afsondret og reguleret miljø. Pladsproblemer skulle dog snart modvirke de gode intentioner."*
- (https://aarhuswiki.dk/wiki/Psykiatrisk_Hospital_i_Risskov)

> Jydske Asyl var i 1800-tallet en af de første moderne psykiatriske institutioner i Danmark og blev fra statens side betragtet som en mønsteranstalt.
>
> Den blev grundlagt på en ny idé om, at sindssygdomme kunne behandles - og ikke opbevares, som det tidligere havde været - og at omgivelserne skulle være rolige, venlige og isolerede.
>
> Asylet kunne rumme op mod 468 patienter i begyndelsen af 1900-tallet, hvor omkring 80 procent af patienterne var fra 3. klasse, som bestod af faglærte og ufaglærte arbejdere.

- DR.DK (https://www.dr.dk/nyheder/indland/indlagt-var-alene-om-kritik-af-sindssygeanstalt-nu-afsloerer-gamle-breve-tingene)

Contextualization of Protocol Entries between *1889 - 1913*
Historical Context

- **By 1889, the hospital had a total capacity of 468 beds.** Yet capacity remained a persistent institutional concern. In 1902, the recently appointed chief physician, Frederik Kristoffer Hallager (1849–1921), publicly reported that the hospital housed 540 patients, exceeding its capacity by 72. That same year, a separate building with space for 14 patients with infectious diseases was constructed.[2]
- Also the procedure of admittance became more () over time; in 1852, the admitting physician would complete a B-form upon patient admission, containing information about the course of the illness and the patient's previous life. The chief physician would review the form with the hospital administration, and decide whether or not the patient should be admitted.[3] To qualify, the illness could not have lasted for more than two years, as patients whose illness had persisted beyond this point were considered incurable (ibid.).
- "Knud Pontoppidan blev i 1898 overlæge for sindssygeanstalten ved Århus. I sin korte tid på stedet iværksatte han flere reformer. Anvendelsen af de lukkede isolationsceller blev afskaffet. Dem havde han tidligere afskaffet på 6. afdeling i København. Dørene blev åbnet. Også fra afdelingerne ud til parken. I stedet for cellerne trådte sengelejet som helbredende foranstaltning. På store overvågede stuer blev patienterne ved indlæggelsen lagt i seng, og de skulle blive der længe. Blev patienterne urolige, kunne de fastspændes med et bælte til sengen."(ibid.)

Administrative Legibility

- definition

---

[2] "Tidslinie for hospitalets forhold," Museum Ovartaci, accessed August 17, 2026, http://www.ovartaci.dk/Default.aspx?ID=1279.
[3] Mus. Ovartaci, "Tidslinie for hospitalets forhold."

Sex and admittance

Diagnoses

- “Emil Kraepelin (1855-1926) opsatte den klassifikation af sindssygdomme, som gjaldt i hele 1900-tallet og delvis stadig danner grundlag for diagnoser. Han skelnede mellem eksogene og endogene psykoser. De eksogene betegnedes som „forgiftninger“ udefra, blandt andre alkoholisme, syfilis og morfinisme. Især betragtede Kraepelin den maniodepressive psykose som endogen. Dvs. noget indefra kommende – noget personen iboende – helt uafhængigt af sjælelivets ydre påvirkninger. Vigtigst var dog hans afgrænsning af de to store endogene sindssygdomme skizofreni (tidligere kaldet dementia præcox) og maniodepressiv psykose. Kraepelin opdagede, at skizofreni indebar en opløsning af personligheden, mens den maniodepressives personlighed forblev intakt. I modsætning til Kraepelin lægger vor tids diagnosesystem, ICD-10 ikke vægt på sindssygdommens årsag og indholdsmæssige sammenhæng” [4] Before then, a sample of diagnoses (exemplified in image 1) would include “psykiske årsager til sindssygdomme” such as:

> · Forsømt, forkuet Barndom
> · Ufornuftig, forkælet Opdragelse
> · Aandelig Overanstrengelse, Examenslæsning, Romanslugeri, Højskolestudier
> · Religiøs Vækkelse
> · Kjærlighedssorger, Jalousi
> · Barnefødsel udenfor Ægteskab
> · Tab af Børn og andre Nærtstaaende
> · Ægteskabelige og huslige Sorger
> · Pengetab, Fallit og lignende Uheld eller Ulykker
> · Næringssorger, økonomisk Tilbagegang
> · Krigsbegivenhederne i 1848-50 og 1864
> · Dagdriveri, ustadigt, planløst Liv [5]

Length of Duration

Literature Review

- Similar research: “Patientstemmer fra asylet” (Klaus Nielsen, Line Keller Ottosen og Signe Düring, 2026) https://unipress.dk/udgivelser/p/patientstemmer-fra-asylet/

c) Expectation / Hypothesis: *present a hypothesis - a tentative ‘answer’ to the question that your investigation will confirm or disconfirm. This way you alert the reader to what is coming.*

The historical organization of the patient records into separate series for women and men raises the question of whether this distinction extended beyond archival organization to patterns of psychiatric classification. The investigation therefore proceeds from the hypothesis that **recorded psychiatric diagnoses were not distributed independently of patient sex, but differed systematically between women and men.** (as contextualized in the background)

RQ1: Did recorded diagnoses differ between women and men?

- ***H_0** Recorded psychiatric diagnosis was <u>independent</u> of patient sex.*

[4] Museum Ovartaci, “12. Sindssygdommenes inddeling,” Psykiatriens historie fra middelalder til nutid, accessed August 17, 2026, https://museum-psyk.dk/psykiatrihistorie/plancher/pl12-indhold.htm.

[5] Museum Ovartaci, “9. Jydske Asyl,” Psykiatriens historie fra middelalder til nutid, accessed August 17, 2026, https://museum-psyk.dk/psykiatrihistorie/plancher/pl9-indhold.htm.

- ***H*₁** *Recorded psychiatric diagnosis was <u>associated</u> with patient sex, resulting in systematic differences in the distribution of diagnoses between women and men.*

Rather than assuming in advance which diagnoses should be associated with either group, the analysis tests whether the observed diagnostic distributions provide evidence against an independent relationship between sex and diagnosis.

A secondary expectation is that these differences cannot necessarily be understood through sex alone. Where the available records permit, the relationship between diagnosis, duration of institutionalization, and recorded outcome provides a possible means of examining whether diagnostic categories were associated not only with admission but also with different institutional trajectories.

RQ2: Were recorded psychiatric diagnoses associated with differences in patients' institutional trajectories?

- *H2: Recorded psychiatric diagnoses were associated with differences in the duration of institutionalization and/or recorded patient outcomes.*

Possible to look at variables: *(modeling)*

- *Diagnosis and gender*
- *Diagnosis + length of duration*
- *Sex + length of duration*
- *Sex + diagnosis + length of duration + (outcome)*
  - *Possible to predict outcome based on these variabled*

<u>Social</u>: occupation/social position → <u>diagnosis OR Gender:</u> Sex → <u>diagnosis</u> OR <u>Length of duration: admission</u> → diagnosis → duration → outcome.

IF TIME AND SPACE:

- Comparative analysis to a contemporary EHR?
- Or look at ***(DEATH)*** ***(https://www.dengamleby.dk/mere-om/udgivelser/aarhus-for-og-nu/kirkegarden-ved-jydske-asyl-psykiatrisk-hospital-i-risskov/) → 'establishing Denmark's first psychiatric hospital cemetery 1857"***

1) Methods

This is a cookbook section detailing how you did your investigation. It provides enough details so that other researchers could replicate your investigation

a) ***Research Design:*** *Briefly explain your overall approach to the research problem (what data did you select to test your question on, what framework are you using, what backdrop are you comparing to, is it a **quantitative nomothetic study or an ideographic analysis** of a given topic? Or a technical pipeline to get some results / make research life easier?)*

Research Design
The present study ()

b) ***Software Framework and Materials:*** *Give a short (one to two sentences) overview of the overall software architecture, including the* ***operating system of your machine, main R (or other) software packages and their versions, and any cloud-based solutions necessary for a successful reproduction of the project results.***

Software Framework:
All analyses were completed using RStudio (v. 4.4.2)

c) ***Data Acquisition and Processing:*** *List and cite all sources of data used in this paper. Justify your motivations in their use and consider their representativeness. Briefly comment on the main steps of your data streamlining and analysis workflow, highlighting decision-making bottlenecks and your decisions (e.g. stopword list construction and removal). Avoid or explain technical jargon. Remember that code goes into the script and not text unless it is part of discussion and GOES BEYOND the scope of coding done in class. Do NOT reproduce the code and functions that you use in the R/Rmd. script unless you are making a major point about it (e.g. modeling, I guess?)*

Data Acquisition
Data was sourced from Rigsarkivets Daisy () *"Protokol over optagne patienter"*. The selected directory includes eight folders of **digitized patient records** between the year 1852 (first year of ()) until 1934 **(fits within the law: accessibility of historical data.)**

Image 1: *Screenshot showcasing available directories*

The **original patient records** consist of (describe the scans) and were first scanned, and thereafter digitized into a (), including both the front- and back pages of each (book). For a limited subset of the directories, the digitized patient records have been transcribed through a "citizen science"-research project at Aarhus University, focusing on conditions under Danish psychiatric admittance between 1895 and 1915 (https://cs.rigsarkivet.dk/collection/13). The project not only includes the patient journals of the present analysis, but also more than 200 private letters written by the patients at the psychiatric hospital in Rosskov, allowing for a view into the patients own anecdotes (see Appendix B1).

The transcriptions were highly structured due to () setting standards for the volunteer-entered annotations (see Appendix B2 and B3).

Data selection

The analysis draws on a subset of the original digital archive, and includes **Rigsarkivet series "Kvinder 1889–1913"** *(Women 1889 - Women 1913)* and **"Mænd 1889–1913"** *(Men 1889 - Men 1913)* from "Psykiatrisk Hospital Risskov: Protokol over optagne patienter." This choice was based on the availability of crowd sourced annotations of the digitized scans being available for both genders during the exact same time period.

Retrieval process

While the transcription of the digitized source was not directly downloadable, the actual website contains repeated structured patient records, presented in sequential order and each represented through the same (variable) fields : (). The structured transcription was not extractable through public APIs, but was stored publicly indexed as HTML. This allowed me to extract the data though without transcribing the corpus manually.

Non-record material was excluded prior to extraction. In both series, the first two digitized images consisted of front matter, meaning that no metadata/annotations were available. Data acquisition therefore began with image 3, the first image containing transcribed patient records. Extraction was bounded by the remaining images within each archival series to prevent navigation beyond the selected collections.

The publicly available transcriptions were computationally extracted into a tabular dataset, with each patient entry represented as one observation and the original transcription fields retained as variables - these were preserved as transcribed. Importantly, genuinely empty transcription fields were distinguished from the annotation *"Kunne ikke udfyldes"* ("could not be completed"). The latter constitutes information introduced during the transcription process rather than an empty field in the historical register (see x). (Indtasters bemærkninger is not provenance metadata in the same sense as the other six. It is a transcription-layer field that isn't displayed in the patient record). Both were therefore retained as distinct states in the raw data, allowing their implications for archival mediation to be considered separately (see Discussion: Archival Neutrality). **Provenance information linking each observation to its transcription page and corresponding digitized archival image was retained to enable verification against the source material.**

Validation against digitized records

Following data extraction, equal-sized samples of the raw male and female patient records were selected, with Løbenr used as the unique record identifier. The female records and one male record were randomly sampled, while Løbenr 4673 was purposely included in the male sample for inspection. The sampled records were downloaded and compared with their corresponding transcribed annotations to manually inspect the correspondence between the digitized source material and the extracted data. This comparison also provided insight into the structure of the material and informed its subsequent standardization. The sampled digitized records are visualized in Appendix C.1.

Data quality assessment and cleaning
Initial Inspection identified several inconsistencies requiring standardization. Entries marked *"Kunne ikke udfyldes"* were standardized to *could_not_transcribe*, preserving them as explicit transcription-status values rather than treating them as missing data, while NA was retained for absent annotations. To examine the distribution of transcription difficulties across the archival structure, occurrences of could_not_transcribe were subsequently counted by field and visualized for fields containing at least 10 occurrences (see Appendix D, Figure D1).

Variables central to the subsequent analyses were additionally subjected to field-specific validity checks. As a first standardization step, all selected categorical variables (except diagnosis_at_admission for analytical purposes) were converted to lowercase, stripped of punctuation, and normalized for whitespace. For variables with a limited set of expected values, including sex, marital_status, and family_disposition, identifiable typographical and transcription variants were subsequently manually recoded when their intended category could be determined unambiguously.

> Skriv hvad der står, som der står, også
> selvom der er åbenlyse stavefejl. Du

A separate analytical dataframe was subsequently derived from the cleaned dataset, allowing records and annotations identified as problematic to be excluded from analysis without altering the cleaned archival data.

Admission, discharge and **birth dates** were inspected for malformed or chronologically inconsistent values. Records with admission years outside the study period (1889–1914) were excluded from the analytical sample, while invalid birth dates were set to NA, as these inconsistencies affected the individual annotation rather than the eligibility of the patient record as a whole. Birth dates later than the corresponding admission year and entries not containing a valid four-digit birth year were treated as invalid.

Finally, patient IDs were inspected for duplicates within each sex-specific series and manually assessed as potential readmissions. Duplicate records that **could not be confidently interpreted as readmissions** were excluded from subsequent analyses and retained separately in /out for documentation and manual inspection. Because the same numeric patient_id could legitimately occur in both the men's and women's series, record identity was defined by the combination of sex_series and patient_id, rather than by patient_id alone. This preserved legitimate occurrences of the same numeric identifier across the two independently organized archival series while allowing within-series duplicates to be identified and evaluated.

Analysis:

Following semi-automated inspections and cleaning, descriptive statistics were performed on the *"diagnosis_at_admission"* Obvious () were standardized and collapses into a separate column, keeping the original transcription as is. More specifically, this (**defend the interpretation of the abbreviations**) was guided by previous literature giving plausible and reliable () (e.g. Kendler)

3) Findings

This section, sometimes headed 'Results', presents the empirical findings of your investigation

a) ***Present the empirical results of your investigation.*** *Describe and explain the counts, statistics, maps or other outcomes (product of your script ~slides, map, outline, timeline…).*

b) ***Illustrate your findings*** *with figures (tables, charts, and graphs) where essential and relevant. Reference these figures in text and describe the most important data as if you were explaining them orally , using a pointer. Your figures and tables must have sufficient information to stand alone, including accurate titles / captions and clear labels for all meaning-carrying feature*

Data collection and structure

The scraping and structuring process yielded *3997* patient records (n women = 1984, n men = 2013) across the two archival series. The records were extracted from 199 scanned pages in the women's series and 203 pages in the men's series, completing a total of 402 archival pages containing structured patient records. The extracted dataset comprised 31 columns, including variables retained or created for computational traceability and subsequent analysis. These consisted of 24 historical record fields displayed in the patient records (NOT FØDSELSDATO), one transcription-specific field (*Indtasters bemærkninger*; 175 non-missing entries), and six extraction and provenance variables *(sex_series, series_page, crowd_id, archive_image_id, transcription_url, and archive_image_api).*

Validation against digitized records

The sampled material consisted of one randomly selected male record (Løbenr 7533), one purposively included male record (Løbenr 4673), and two randomly selected female records (Løbenr 5106 and 7713). For each record, the transcribed annotation was retained alongside its corresponding URL linking to the digitized patient record.

Manual inspection revealed several inconsistencies between the **digitized patient records** and their **transcribed annotations**. For example, the transcribed annotation for one patient record (Løbenr 4673) contained an invalid date, rendered as 1897-01-32. Comparison with the corresponding digitized record (see Image 2) showed the handwritten entry to be 21 January 1897, indicating that the invalid value was introduced during transcription. The example illustrates not only the need for data cleaning prior to computational analysis, but also how the process of transcription can alter the digital representation of archival material. Such discrepancies complicate the treatment of digitized and transcribed archival material as neutral or exact reproductions of the historical source (see Archival Neutrality for further discussion).

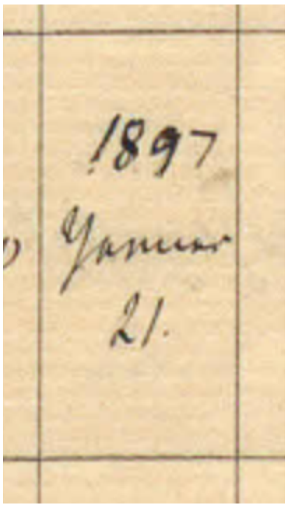

Image 2: Record image from https://cs.rigsarkivet.dk/picture/view-values/1533175 *"Johan Vilhelm Pahl"* (Løbenr: 4673)

The columns in the **original patient records** were (concsistently? inconsistently?) structured
Data quality assessment and cleaning results

Final analytical sample
Across the 3997 extracted records, 3977 unique patient numbers were identified. 21.271 cells within the transcribed records were unidentifiable in the digitized- or missing in the original patient records.

- Number of patients with inconsistent admission/discharge dates: 32

Additionally, "Fødselsdato" (date of birth) was often added retrospectively at the top of the cell designated "Alder ved Indlæggelsen" (age at admission), constituting a later, non-standardized addition to the original record structure. Of the eligible records, 1,697 contained a transcribable birth-date annotation. Such annotations became consistently present among records from 1901 onward, with a single earlier instance in 1893 (see Appendix C, Table C2).

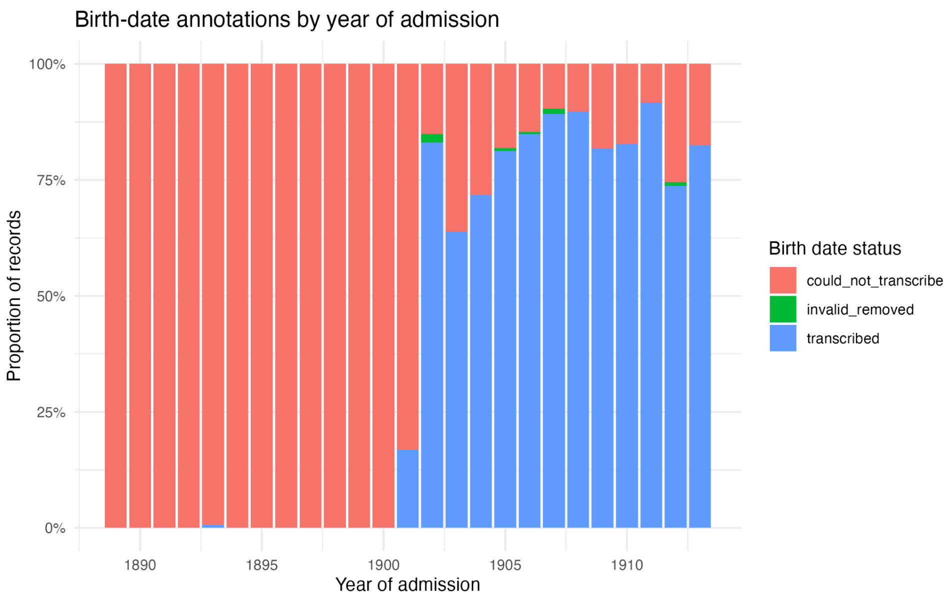

Image 3:

Descriptive Statistics
From the transcriptions, some non-standardized variables were highly heretogeneous, including *condition_at_discharge* (n = 219 unique values) *and diagnosis_at_admission* (n = 543 unique values).

Following standardization, *diagnosis_at_admission (n = unique values).*

Ikke sindssyg	12

Dem præc = "dementia praecox" [6]

4) Discussion

This is the main part of the report , the part that will be read with the most care by other historians /professionals. Here you explain the significance of your findings by relating what you discovered to the problem you set out to investigate in your introduction.

a) ***Significance of findings*** *Return to the original research question/problem and relate your results to it. What do your results mean in the context of your research premise?*

b) ***Evaluate****: Did your investigation accomplish your purpose? Did it answer your questions? Were you able to confirm or disprove your original assumptions/expectations? How do they compare to your comparanda? (historical background, data reliability and representativeness)?*

While (), approved transcription is not synonymous with error-free ground trust. **(INCLUDE REF TO RESULTS SECTION)**

The original structure is in- and of itself historical: The institution has decided which characteristics of a patient are sufficiently important to record..

- **Archival Neutrality:** examples of **new metadata produced during digitization → Digitization:**
- Annotations were almost always transcribed as they were written, and it required close reading and comparative analyses to recognize whether these were typographical mistakes by the annotator or simply misspellings in the original patient journals.
    - As a digital practitioner, I am adding another layer to the structurization of this: *I* decide how historical categories that R can manipulate.
    - Crowdsourced transcriptions: caveats and good stuff

c) ***Consider Utility/Representativeness*** *Are your results useful? Why or why not? (Are your data and results representative or limited in certain ways?)*

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[6] Kenneth S. Kendler, "The Development of Kraepelin's Concept of Dementia Praecox: A Close Reading of Relevant Texts," *JAMA Psychiatry* 77, no. 11 (2020): 1181–87, https://doi.org/10.1001/jamapsychiatry.2020.1266.

d) ***Lessons learnt / New avenues*** *Did you discover information that you hadn't anticipated? Was your research design appropriate? Did your investigation raise new questions? Are there implications from your results that need to be explored? (bigger sample needed, comparison needed, more representative dataset needed…*

-

5) Conclusions and recommendations. *What you say in this section depends on the context of your investigation and the expectation of your readers.*

a) **Summarize the most important findings. Focus and reflect on the main lessons learnt while working on the project.**

Ethical Considerations:
Permission was given by ()

Acknowledgements: Rigsarkivet….

References

Appendices

A: Required Metadata

Table A1: Software Metadata

Software Metadata	Description
Link to Github repository	
Software License	
Data License	
Software versions, Installation requirements & dependencies for software not used in class	
Link to software documentation for special software (only relevant if you go outside the scope of class)	
Support e-mail for questions	*au.matildeelene@gmail.com*

Table A2: Data Metadata

Metadata	Description
concat_1889_1913_raw.csv	*Global smoking statistics among youth in 2000-2020 collected by the World Health Organisation, extracted from OurWorldinData on 10 October 2020 with five columns: country, year, gender, age, count* *With 24 original, extracted columns: Løbenr, Navn, Køn, Stand og stilling, Hjemsted, Alder ved indlæggelsen, Fødselsdato, Trosbekendelse, Ægtestandsforhold, Naar ankommen, Hvorfra ankommen, Diagnose ved indlæggelsen, Varighed ved indlæggelsen, Journalnummer, Forplejnings og betalingsklasse, Anfaldets nr., Anmærkninger, Familiedisposition, Afgået nr., Afgået naar, I hvad tilstand, Hvorhen, Ældre nr.,* and *Genindlagt under nr.]* *An additional "Indtasters bemærkninger"* and seven additional columns: *['sex_series', 'series_page', 'crowd_id', 'archive_image_id', 'transcription_url', 'archive_image_api']*

B: Patient Record Metadata

B1: Image showing purpose of annotation

Om Patientjournaler fra Risskov

Psykiatriens historie har hidtil primært været skrevet og fortalt af lægerne og psykiaterne, men i de senere år har der været en fornyet interesse i, også at inddrage patienternes oplevelser af dét at være indlagt på de psykiatriske institutioner, i historieskrivningen.

Et nyt forskningsprojekt ved Århus Universitet vil forsøge at afdække forholdene i dansk psykiatri mellem 1895 og 1915 ved at tage udgangspunkt i patienternes egne fortællinger, blandt andet belyst gennem flere end 200 private breve, skrevet af patienter på det psykiatriske hospital i Risskov, og gennem patientjournalerne, der opbevares i Rigsarkivets samlinger.

Forskningsprojektet vil undersøge, om patienternes sociale og økonomiske forhold havde en betydning for den diagnose de fik, hvad klassetilhørsforhold betød for den meget høje dødelighed, der var blandt institutionens patienter, og hvordan patienternes perspektiver påvirkede etableringen af de moderne psykiatriske institutioner i Danmark.

Denne indtastningsvejledning er udviklet i samarbejde med forskningsprojektet, slægtsforskere og Rigsarkivet.

B2: Annotation Status

Psykiatrisk Hospital Risskov: Protokol over optagne patienter

Beskrivelse	
Omfang	Dokumenter: 612, Godkendte: 612
Gennemført	Patientprotokol 100%
Projekt	Psykiatrisk Hospital Risskov

Bliv indtaster

Indhold

Vis kun serier klar til indta: Filtrer

Navn	Patientprotokol Klar til indtastning	Patientprotokol Afventer korrekturlæsning	Godkendte Patientprotokol	Dokumenter
Kvinder 1889 - Kvinder 1913	0	0	207	207
Mænd 1861 - Mænd 1889	0	0	196	196
Mænd 1889 - Mænd 1913	0	0	209	209

Tilbage

B3: Annotation Guideline

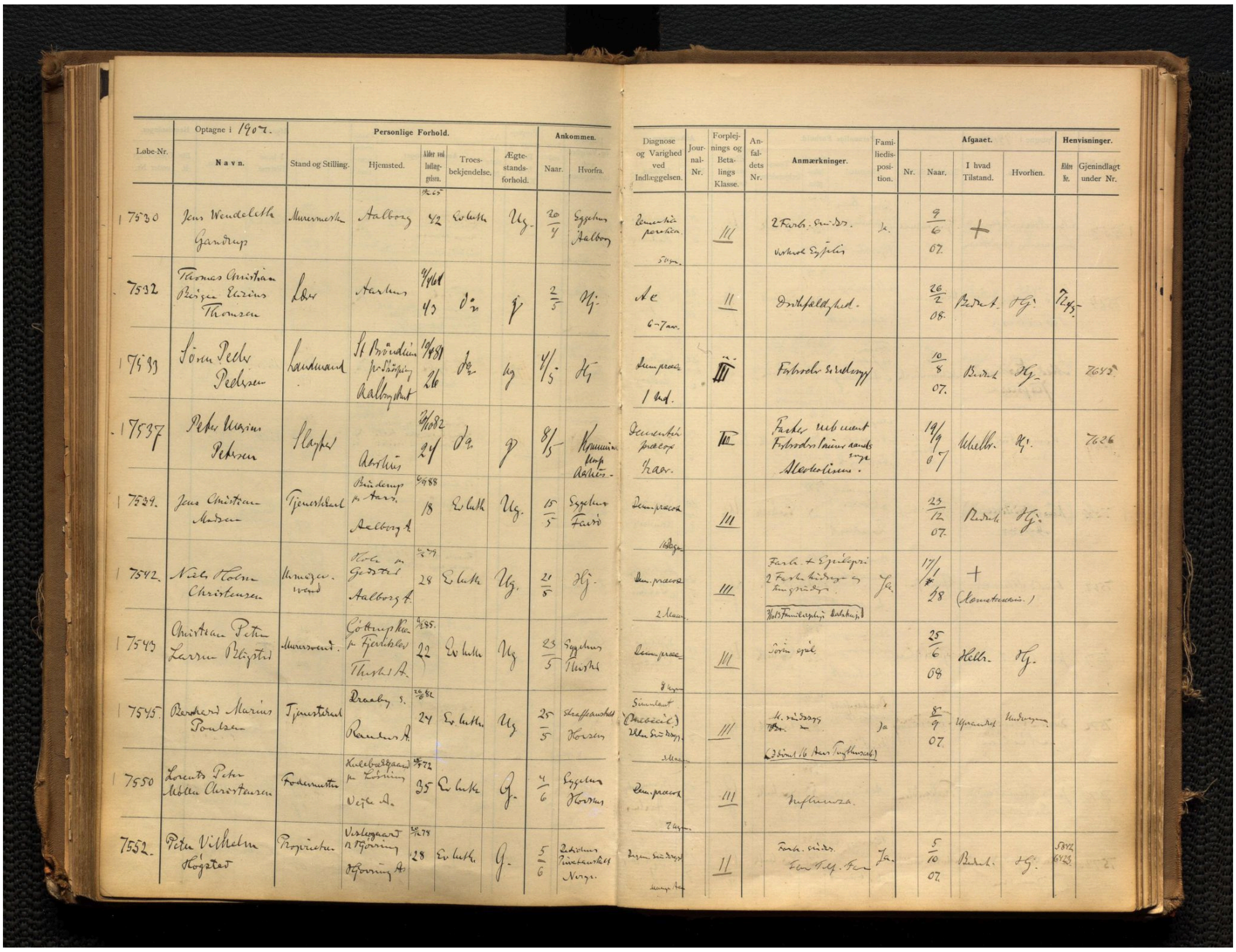
- **UPLOAD PDF**

C: For Comparative Analysis (Manual Validation)

C1: Randomly drawn images*

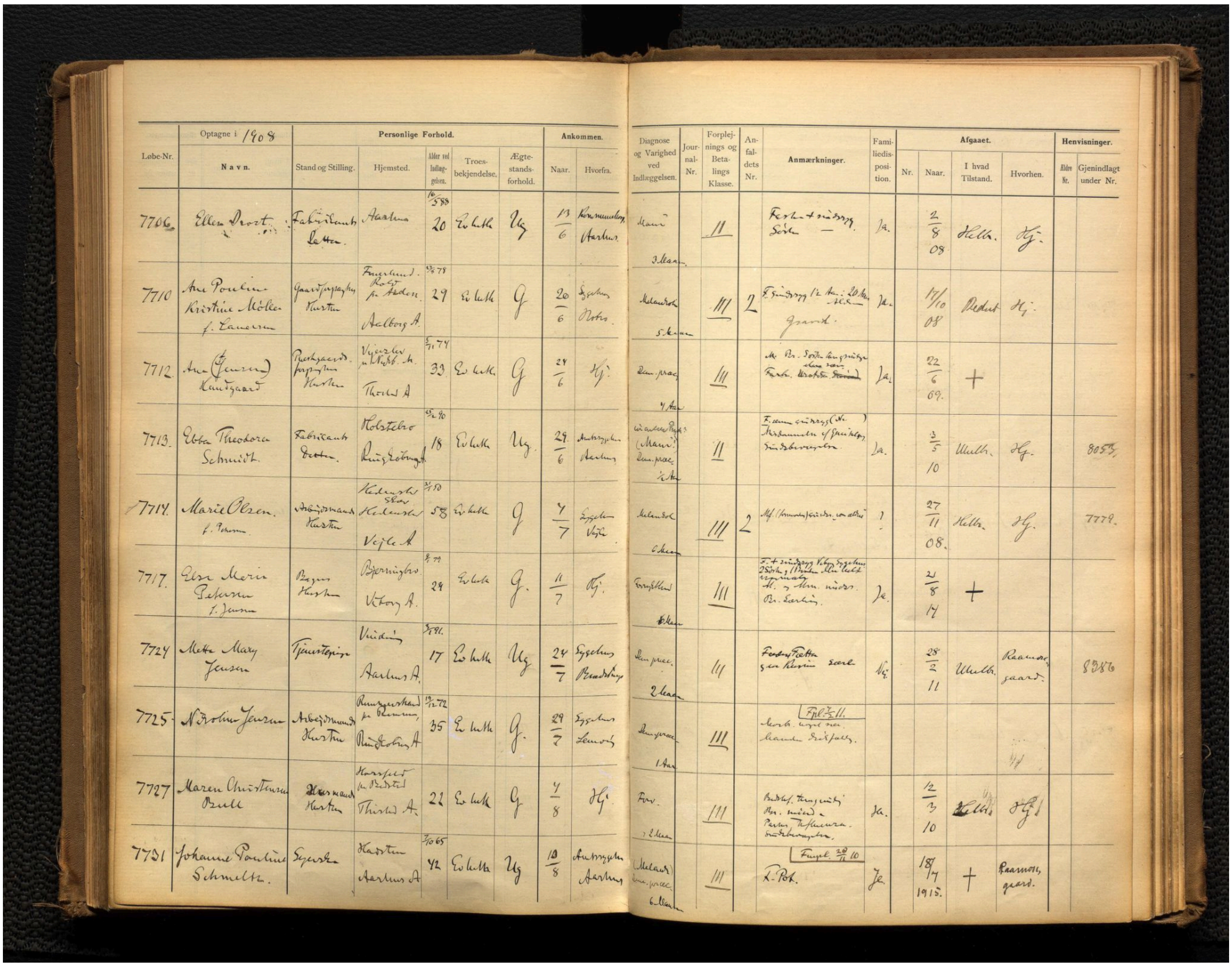
Løbe-Nr.	Optagne i 1905. Navn.	Stand og Stilling.	Hjemsted.	Alder ved Indlæggelsen.	Troesbekjendelse.	Ægtestandsforhold.	Ankommen. Naar.	Ankommen. Hvorfra.	Diagnose og Varighed ved Indlæggelsen.	Journal-Nr.	Forplejnings og Betalings Klasse.	Anfaldets Nr.	Anmærkninger.	Familiedisposition.	Afgaaet. Nr.	Afgaaet. Naar.	I hvad Tilstand.	Hvorhen.	Henvisninger. Ældre Nr.	Gjenindlagt under Nr.
7232.	Poul Bekker Hansen	Handelslærling.	Ribe.	3/10 88. 17	Ev. luth.	Ug	17/11.	Privat Familie Aarhus.	Slövsind 2 Aar		II		M. hysterisk	Ja.		16/2 06	Bedret.	Hj.		
7234.	Klemens Thomsen	Arbejdsmand	Hillerslev pr. Thisted. Thisted A.	28/? 72. 33.	Ev. luth.	Ug.	22/11	Hj.	Meland. 6 Maan.		III	2	Bedstemoder ag. tungsindig. Br. Pt. 6833 Chr. Thomsen pr. [illegible]			19/10 06	Helbr.	Hj.		
7235.	Niels Sørensen Amstrup	Mejerist	Ørst pr. Randers. Randers A.	10/12 85. 19	Ev. luth.	Ug	23/11.	Sygehus Randers.	D. praec. Slövsind 2 Maan.		III		F. og Bf. tungsindige. Onkel Potator.	Ja.		2/6 06	Bedret	Hj.		
7236.	Christen Pram Pedersen kld. Østergaard.	Slagteriarbejder	Horsens. Aarhus A.	?/12 68. 36	Ev. luth.	G.	30/11	Hj.	Forrykthed 5 Maan.		III		M. nervøs	Ja.		22/3 09	Uhelbr.	Dalstrup		
7237.	Lauritz Christensen Hougesen	Sadelmagersvend	Viveld pr. Allingaabro. Randers A.	9/6 75. 30	Ev. luth.	Ug	30/11	Privat Forplejning.	Slövsind 9 Aar		III					14/5 06	uhelbr.	Dalstrup	6287 6994	7854.
7238.	Søren Rasmussen Hvid.	Arbejdsmand	Aarhus.	11/8 76. 29	Ev. luth.	G.	5/12	Kommunehospit. Aarhus.	Forrykthed. 1 Maan		III	3	Mf. Pot. [illegible]. M. [illegible] Mor + Apoplexi [illegible]	Ja		4/5 19.	Uhelbr.	[illegible] bort fra Dalstrup	6219.	
7240.	Frederik Nielsen Vejlstrup	Agent	Aalborg	?/10 51. 54.	Ev. luth.	G.	8/12	Sygehus Aalborg	Slövsind D. paralyt. (1 Maan)		III					22/3 06	+			
7243.	Niels Christian Nielsen	Arbejdsmand	Tversted pr. Sindal. Hjørring A.	66	Ev. luth.	G.	13/12	Sygehus Hjørring.	Forrykthed 5 Aar.		[illegible]		Potator.			7/9 06	Uhelbr.	Dalstrup		8257.
7245.	Thomas Christian Birger Elisius Thomsen	Lærer	Aarhus	3/4 64. 41.	Ev. luth.	G	19/12	Hj.	Ak. 5 Aar		III		[illegible]			12/12 06	Bedret	Hj.		7532.
7246.	Andreas Marius Jensen Nygaard	Avlsbrugers Søn	Viborg Mark. Viborg A.	14/6 86. 19	Ev. luth.	Ug	19/12	Amtssygehus Viborg	D. praec. Slövsind 2½ Aar		III		[illegible] M. nervøs; [illegible] M. [illegible]	Ja		2/6 07.	+			

Image C1.1: (Løbenr:)



Løbe-Nr.	Optagne i 1907. Navn.	Stand og Stilling.	Hjemsted.	Alder ved Indlæggelsen.	Troesbekjendelse.	Ægtestandsforhold.	Ankommen. Naar.	Hvorfra.	Diagnose og Varighed ved Indlæggelsen.	Journal-Nr.	Forplejnings og Betalings Klasse.	Anfaldets Nr.	Anmærkninger.	Familiedisposition.	Afgaaet. Nr.	Naar.	I hvad Tilstand.	Hvorhen.	Henvisninger. Ældre Nr.	Gjenindlagt under Nr.
7530	Jens Wendelelth Gandrup	Murermester	Aalborg	19/2 65 42	Ev. luth.	Ug.	20/4	Sygehus Aalborg	Dementia paralytica. 5 Uger.		III		2 Farbr. sindss. Vistnok Syfilis	Ja.		9/6 07.	+			
7532	Thomas Christian Birger Elarius Thomsen	Lærer	Aarhus	9/4 64 43	do.	G.	2/5	Hj.	Ae 6–7 aar.		II		Drikfældighed.			26/2 08.	Bedret.	Hj.	7245.	
7533	Søren Peder Pedersen	Landmand	St Brøndum pr Skørping Aalborg Amt	10/4 81 26	do.	Ug.	4/5	Hj.	Dem. praec. 1 Md.		III		Farbroder sindssyg			10/8 07.	Bedret	Hj.		7645.
7537	Peter Marius Petersen	Slagter	Aarhus	21/10 82 24	do.	G.	8/5	Kommune Hosp. Aarhus.	Dementia praecox 1 aar.		III		Fader ubekendt Farbroders Hustru aandssvag Alcoholismus.			19/9 07	Uhelbr.	Hj.		7626
7539.	Jens Christian Madsen	Tjenestekarl	Bindeup pr. Aars. Aalborg A.	6/9 88 18	Ev. luth.	Ug.	15/5	Sygehus Farsø	Dem. praecox 1 Dage.		III					23/12 07.	Bedret	Hj.		
7542.	Niels Holm Christensen	Urmagersvend	Hole pr. Gadstrup Aalborg A.	9/2 79 28	Ev. luth.	Ug.	21/5	Hj.	Dem. praecox 2 Maaned.		III		Farbr. + Epilepsi 2 Farbr. hidsige og tungsindige. 3/10 13 Familiepleje Dalskov	Ja.		17/1 28	+ (Konstateret.)			
7543	Christian Peter Larsen Bligsted	Murersvend.	Göttrupkær pr Fjerritslev Thisted A.	9/2 85. 22	Ev. luth.	Ug.	23/5	Sygehus Thisted	Dem. praec. 8 Uger.		III		Søstre syg.			25/6 08	Helbr.	Hj.		
7545.	Reinhard Marius Poulsen	Tjenestekarl	Draaby s. Randers A.	26/6 82 24	Ev. luth.	Ug	25/5	Straffeanstalt Horsens	Simulant (Imbecil) Uden Sindssyg. 3 Maan.		III		M. sindssyg Br. — (Idømt 16 Aars Tugthusarb.)	Ja		8/9 07.	Uforandret	Undersøgt		
7550	Lorents Peter Møller Christensen	Fodermester	Kulebøllgaard pr. Lørning Vejle A.	18/5 72 35	Ev. luth.	G.	4/6	Sygehus Horsens	Dem. praecox 7 Uger.		III		Influenza.							
7552.	Peter Vilhelm Høgsted	Proprietær	Vestergaard pr Hjørring Hjørring A.	20/11 78 28	Ev. luth.	G.	5/6	Sindssyge Privatanstalt Norge.	Ingen Sindssyg.		II		Farbr. sindss. [illegible]	Ja.		5/10 07.	Bedret.	Hj.	5842 6423	

Image C1.2:



Løbe-Nr.	Optagne i 1908 Navn.	Stand og Stilling.	Hjemsted.	Alder ved Indlæggelsen.	Troesbekjendelse.	Ægtestandsforhold.	Ankommen. Naar.	Hvorfra.	Diagnose og Varighed ved Indlæggelsen.	Journal-Nr.	Forplejnings og Betalings Klasse.	Anfaldets Nr.	Anmærkninger.	Familiedisposition.	Afgaaet. Nr.	Naar.	I hvad Tilstand.	Hvorhen.	Henvisninger. Ældre Nr.	Gjenindlagt under Nr.
7706.	Ellen Drost.	Fabrikants Datter.	Aarhus	16/5 88 20	Ev. luth.	Ug	13/6	Kommunehosp. Aarhus.	Mani 3 Maan.		II		Farbr. + sindssyg Søster —	Ja.		2/8 08	Helbr.	Hj.		
7710	Ane Pouline Kristine Møller f. Lauersen	Gaardforpagters Hustru	Fuglelund. Rold pr. Arden. Aalborg A.	23/8 78 29	Ev. luth.	G.	20/6	Sygehus Hobro.	Melancholi 5 Maan.		III	2	F. sindssyg 1/2 Aar: 20 Aar Alder graad.	Ja.		17/10 08	Bedret	Hj.		
7712.	Ane (Jensen) Hundgaard	Præstegaardsforpagters Hustru	Vigerslev pr. Nykøb. M. Thisted A	5/7 74 33	Ev. luth.	G	24/6	Hj.	Dem. praec. 4 Aar		III		M. Br. Søster tungsindige [illegible] Farbr. [illegible]	Ja.		22/6 69.	+			
7713.	Ebba Theodora Schmidt.	Fabrikants Datter.	Holstebro Ringkøbing A.	15/2 90 18	Ev. luth.	Ug.	29/6	Amtssygehus Aarhus	[illegible] (Mani) Dem. praec. 1/2 Aar		II		F. ikke sindssyg (Nr.) Misdannelse af Gebisset, Sindsbevægelse	Ja.		3/5 10	Uhelbr.	Hj.		8053.
7714.	Marie Olsen. f. Pedersen	Arbejdsmands Hustru	Kielstrup Skov Kielstrup Vejle A	21/1 50 58	Ev. luth.	G	4/7	Sygehus Vejle	Melancholi 6 Maan.		III	2	Mf. (Morbror) sindssyg, [illegible]	?		27/11 08.	Helbr.	Hj.		7779.
7717.	Else Marie Petersen f. Jensen	Bagers Hustru	Bjerringbro Viborg A.	8/1 79 29	Ev. luth.	G.	11/7	Hj.	Forvirthed 6 Maan.		III		F. + sindssyg Viborg Sygehus 2 Søstre, 1 Broder ikke helt normale M. og Mm. nervøs. Br. Særling.	Ja.		21/8 17	+			
7724	Mette Mary Jensen	Tjenestepige	Vinding Aarhus A.	3/1 91. 17	Ev. luth.	Ug	24/7	Sygehus Brædstrup	Dem. praec. 2 Maan.		III		Fader Fætter gen. Kusine svag.	Nej.		28/2 11	Uhelbr.	Raamosegaard.		8386
7725.	Nikoline Jensen	Arbejdsmands Hustru	Ringgaard pr. Lemvig, Ringkøbing A	13/12 72 35	Ev. luth.	G.	29/7	Sygehus Lemvig	Dem. praec. 1 Aar		III		Fpl. 7/2 11. Mødr. ugift nervøs Moder drikfældig.							
7727	Maren Christensen Bull	Husmands Hustru	Hørsted pr. Bedsted Thisted A.	22	Ev. luth.	G	4/8	Hj.	For. 2 Maan.		III		Bedstef. tungsindig. Br. nervøs, Faster Influenza. Sindsbevægelse.	Ja.		12/3 10	Helbr.	Hj.		
7731	Johanne Pauline Schmeltz.	Syerske	Harstrup Aarhus A	3/10 65 42	Ev. luth.	Ug	10/8	Amtssygehus Aarhus	(Melanch.) Dem. praec. 6 Maan.		III		Fampl. 28/11 10 L-Pot.	Ja.		18/7 1915.	+	Raamosegaard.		

Image C1.3:

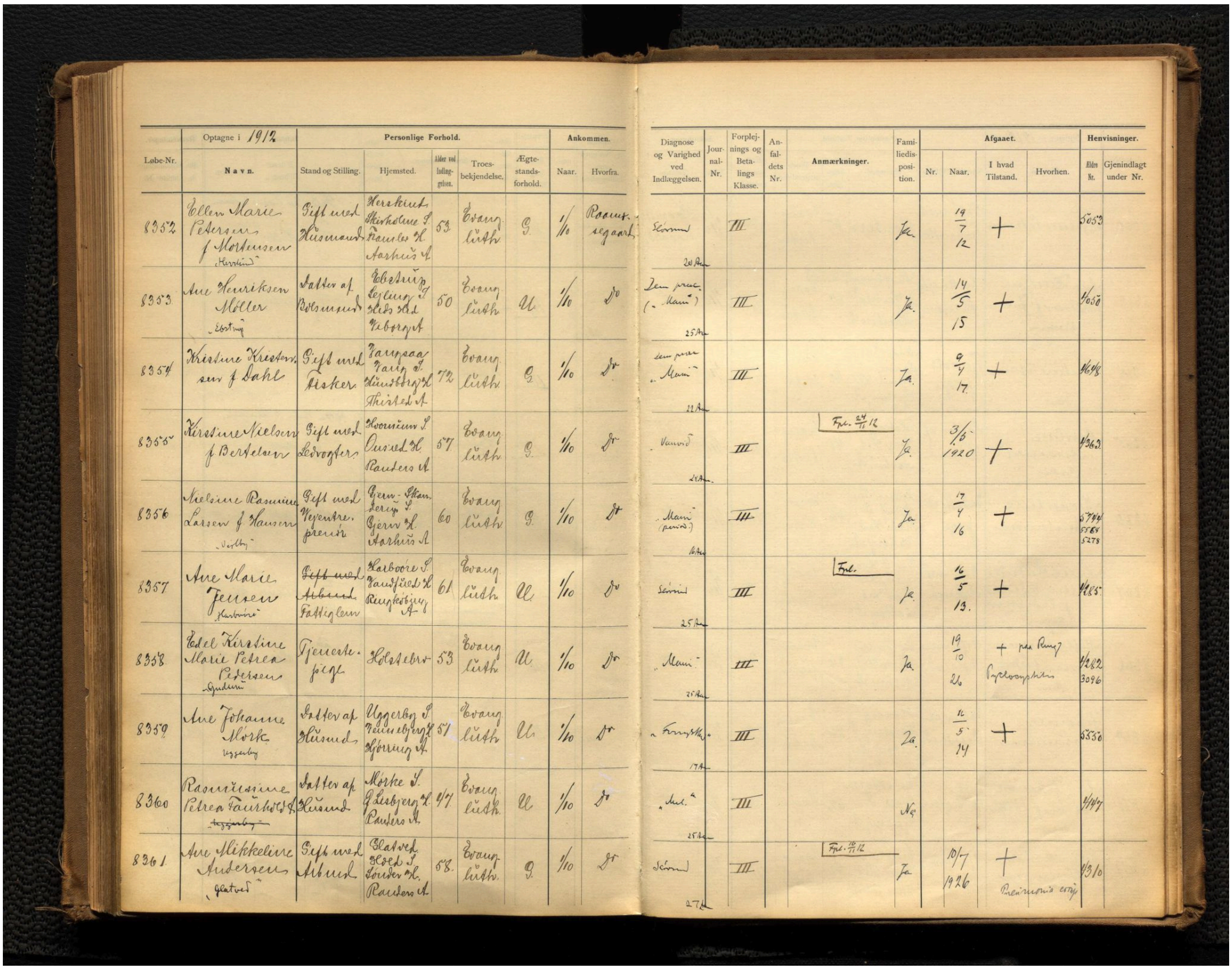
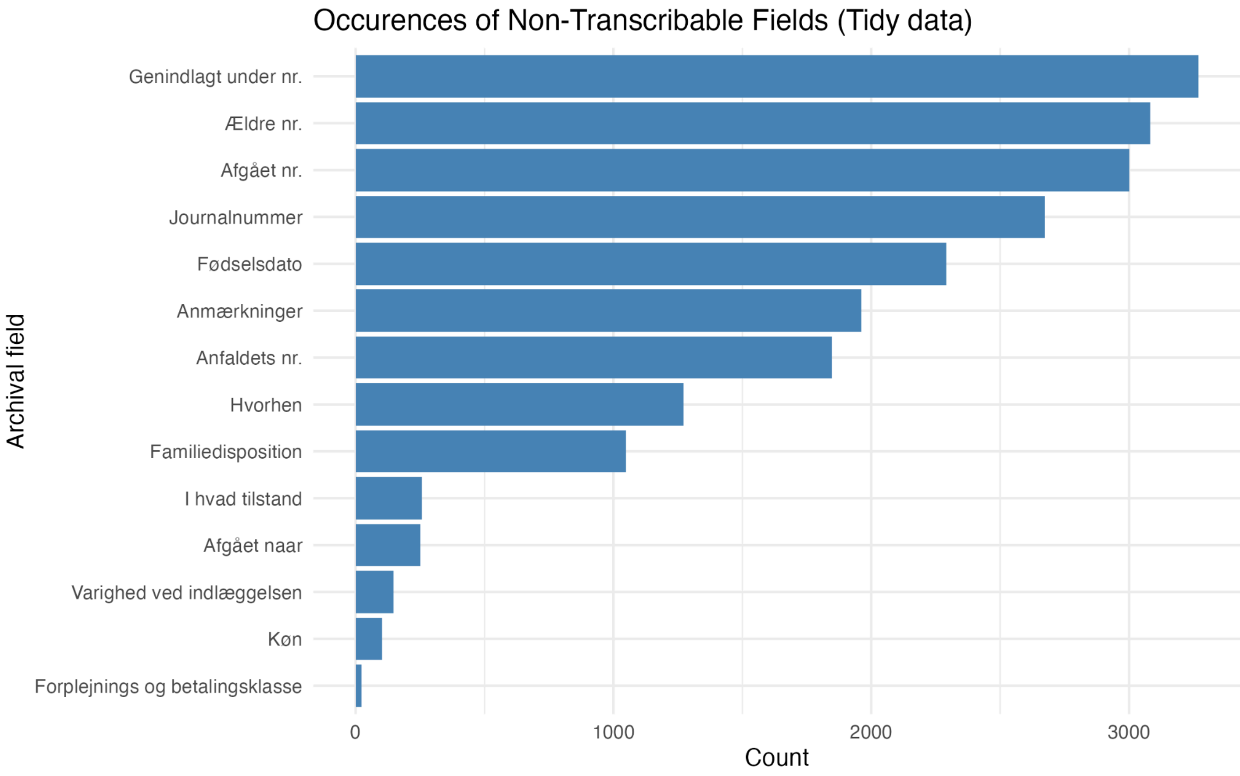


Image C1.4:

**The corresponding transcribed datasets are saved as csv files in out/digitized_comparison.*

Appendix D: Descriptive Analysis Outputs

Figure D1: Visualization of archival fields including “could_not_transcribe”


Occurences of Non-Transcribable Fields (Tidy data)
Archival field
Genindlagt under nr.
Ældre nr.
Afgået nr.
Journalnummer
Fødselsdato
Anmærkninger
Anfaldets nr.
Hvorhen
Familiedisposition
I hvad tilstand
Afgået naar
Varighed ved indlæggelsen
Køn
Forplejnings og betalingsklasse
0
1000
2000
3000
Count